VALLEY REHAB & PHYSICAL THERAPY - INITIAL EVALUATION
Patient: Morgan Ellis DOB: 04/11/1985
Date of Evaluation: 01/20/2026 Therapist: Priya Nair, PT, DPT
Referring Provider: Alan Brooks, MD (Northgate Family Medicine)
Diagnoses: Cervical strain S13.4XXA; Lumbar strain S39.012A
Date of Injury: 01/09/2026 (rear-end motor vehicle collision)

SUBJECTIVE:
Patient reports constant neck pain, currently 7/10, worst 9/10 at night, best 5/10 after medication.
Low back pain 5/10, worse with prolonged sitting and bending. Headaches daily, lasting 2 to 4 hours.
She reports difficulty sleeping, turning her head to check blind spots while driving, carrying laundry,
and lifting her 3-year-old son. She has been off work since the collision. She reports feeling anxious
as a passenger and avoids the freeway. Prior level of function: independent and pain free, working
full time, exercising at the gym 3 times per week.

OBJECTIVE:
Posture: Forward head, elevated left shoulder, guarded cervical movement.
Cervical AROM: Flexion 30 degrees (normal 50). Extension 25 degrees (normal 60). Rotation right 50
degrees, left 40 degrees (normal 80). Side bend right 20 degrees, left 15 degrees (normal 45).
Lumbar AROM: Flexion 50% of normal with pain. Extension 25% of normal with pain.
Palpation: Marked muscle guarding of the left upper trapezius, levator scapulae, and suboccipitals.
Lumbar paraspinal hypertonicity L3-L5 bilaterally.
Strength: Deep neck flexor endurance 8 seconds (normal greater than 30).
Neurological screen: Sensation intact. No upper limb tension signs at this time.

OUTCOME MEASURES:
Neck Disability Index: 44% (severe disability).
Oswestry Disability Index: 32% (moderate disability).
Patient-Specific Functional Scale: lifting son 2/10, driving 3/10, sleeping through the night 2/10.

ASSESSMENT:
Patient presents with signs and symptoms consistent with cervical and lumbar strain following a rear-end motor vehicle collision on 01/09/2026, with significant limitations in mobility, sleep,
self-care, child care, driving, and work. Her deficits are consistent with the mechanism of injury.
Rehab potential is good with skilled intervention.

GOALS (8 weeks):
1. Reduce neck pain to 3/10 or less with daily activities.
2. Improve cervical rotation to 70 degrees bilaterally for safe driving.
3. Lift 30 pounds floor to waist without increased pain, to lift her son and return to work.
4. Neck Disability Index below 20%.

PLAN:
Physical therapy 3 times per week for 8 weeks: manual therapy, therapeutic exercise, neuromuscular
re-education, modalities as needed, and a home exercise program.

Signed: Priya Nair, PT, DPT 01/20/2026